Six Years' Experience with LSD Therapy

Fred W. Langner, M.D.*

This paper consists of three parts, and samples two periods covering a total of nearly six years' experience with LSD therapy. The first period, 1954-57, will be summarized from a paper I wrote in 1957. This paper was a means of taking inventory of the pros and cons for continued use of LSD therapy. This evaluation of my own impressions led to an exciting consideration, namely LSD as *modus operandi* for therapeutic self-examination in the non-psychotic.

PART ONE

My initial concern with LSD derived from the observation that the discipline of analytically-oriented psychotherapy is prohibitive for many patients because of its financial and temporal demands, and a shortage of therapists. Yet there has been no other mode of therapy adequately experiential to resolve conflicts and foster emotional growth. In the interest of determining if lysergic acid reactions as reported by Sandison (1) might offer a means of effectively telescoping analytic benefits into a short-term experience, the potentialities of these reactions were explored. During a period of more than two years (1954-57) sixty patients (representing most diagnostic labels but preponderantly schizophrenia and severe personality disorders) were observed while under the influence of lysergic acid during more than 600 episodes. For most of these patients, it was the major therapeutic approach.

In this report a general evaluation of its effectiveness will be attempted. It is accepted, *a priori,* that the evaluation is a non-controlled study and to a considerable degree subjective. The initial cases had a poor prognosis based on their chronicity and history of previous treatment failure. The encouraging results with these caused me to enlarge and continue the study.

Lysergic acid was initially defined as a toxic agent which induces a "model psychosis." My objections to this concept will be presented later. For the present my concept of the drug as a therapeutic agent is defined

* Director Langer Clinic, Albuquerque, New Mexico.

as follows: Lysergic acid is an agent which diminishes effective compartmentalization within the mind, attenuates and often shatters the multiple defensive adaptations which underlie neurotic behavior, permits in many patients heavily repressed experiences to be brought rapidly into consciousness and, in general, catalyzes the revocation and comprehension of the traumatic roots of emotional illness. I have come to think of the drug as a mental and emotional catalyst and an ego-enhancing agent. It is from this standpoint that the drug is discussed in this paper.

Method of LSD Therapy

Each patient covered in this report of 1954-57 was handled, as regards his LSD therapy, in essentially the same manner. Ninety to one hundred and twenty minutes after a normal breakfast, the patient was given an oral dose of LSD in a glass of water. Dosages varied from 0.4 to 3.0 cc (40 to 300 mcg). An attendant, functioning in an entirely permissive manner and with duties chiefly as observer and stenographer, was with the patient continuously. The doctor was in and out intermittently but tried to be present whenever the assisting therapist thought the situation was noteworthy. A record of the patient's verbalizations, behavior and affect was kept in as full detail as possible. The duration of each LSD episode averaged four to six hours. When it was deemed necessary (to preserve the patient from too threatening disorganization), the reaction was ameliorated or terminated with intravenous barbiturate or Frenquel, or, rarely, both. Moderate oral sedation and Frenquel were used routinely for a while to taper the reactions, but were found to be unnecessary as a routine procedure. In some episodes, intravenous methedrine or methedrine-barbiturate were used to accelerate or modify existing responses.

Rate of Catharsis

Noteworthy is the appearance in some cases, during the first few episodes, of the symbolic or literal expression of many of the dynamic factors involved in the illness. These cases illustrate how the disinhibiting aspect of the drug may permit a frontal attack on problems, even after formal interviews had brought comparatively unrewarding results.

A patient who was treated unsuccessfully elsewhere with electroshock therapy for a severe neurotic depression was admitted to the sanatorium after a recurrence of a suicidal attempt. He was uncommunicative for the first week. During the second week, he was verbalizing moderately freely, but during his fourth formal interview, he became resistant to the conference procedure, protesting that the doctor's concern with the past was purposeless as it was the present he had to face when he went home. This resistance followed the first admission of mutual masturbation as a boy and his continued need for self-masturbation in his present marriage. The next day he was bed-ridden, hysterically withdrawn, and hinting at suicide. After ten days of highly regressive and negative behavior, he reestablished contact and within the following week was able to reveal current mutual masturbation with an older man and his feeling of irrita-

tion with his marital heterosexual relationship. Thus, in over a month's relationship and hospitalization, little had been revealed verbally beyond an expression of the present focal symptom of overt homosexuality.

At this point, LSD was introduced. The initial dose (100 mcg, to which 100 mcg were added after one hour) induced relatively little response. Three days later, in response to 300 mcg, the following material was freely verbalized.

1. Feelings of rejection by father, and its equivalence to castration.
2. Ambivalence toward the father, including overt death wishes,
3. Feelings toward oldest brother, similar to, but not as acute as, those involving father.
4. Unsuccessful attempt to compete family-wise with youngest sister.
5. Awareness of pathologic dependence on father and structuring of this relationship as a scaffolding for projection.

During the next episode, two days later, the patient experienced being incarcerated and crushed in the womb. He reacted as if the experience were intolerably painful, physically and emotionally. To alleviate the distress without sacrificing alertness, a combination of barbiturate-methamphetamine was given intravenously. Shortly thereafter the following conversation was recorded:

Doctor: Why do you think you seemed to stop in your mother's womb?
Patient: My father. . . . You (the doctor) take your children to the beach, hikes, picnics and to things. . . .
Doctor: Are you out of the womb yet?
Patient: No, Siree.
Doctor: Why?
Patient: I don't know, but I believe that people told me that I should not have been born.
Doctor: What do you want most?
Patient: To be born physically; to be wanted; to be loved.
Doctor: What do you expect from your father and mother?
Patient: My father wouldn't do the right thing by me. He shouldn't have showed my brothers all his love and attention.
Doctor: What does being born represent to you?
Patient: You mean at birth?
Doctor: Yes.
Patient: They wanted so much for me to be a baby girl at birth. And I've tried so hard. I've washed dishes, cleaned the house. I've been a girl so long that there's been nothing left tc wish.
Doctor: Have you ever wished that you didn't have the genital organs of a male?
Patient: Yes—even to the point of cutting them off if it wasn't for the pain. . . .

Following this, in the doctor's absence, the patient demanded that the doctor undertake to pull him out of the womb since he could not get out by himself. Then he revealed that the feeling of being crushed was not from the feeling of being delivered but was a feeling of being crushed by

his father while still in the womb and that he hated him for having sex relations with his mother while he was in there. Later in the episode, this recollection from childhood occurred:

> "Patient remembered that from the first time he ever heard a chicken in the shell he couldn't resist the temptation to tear the shell open and let the chicken out. He says that he was afraid that when the hen sat on the eggs she would mash it to death before the baby chick could get out of the shell by itself."

Four days later, the patient experienced his fourth LSD episode. He found himself back in the womb with his father's penis painfully jabbing his right eye—the same area referred to in his pre-LSD interviews as the site of frequent, severe headaches when he fears he has lost friends. During this LSD episode, the patient spontaneously evolved his sexual development in reference to his father as follows: watching his father urinate stimulated the sex urge in him. This was before he started masturbating. During puberty, he started having ejaculations and he wanted desperately to talk to his father about his desire to have relations with him. If he had mentioned this, his father would have beaten him brutally. Sexual rejection by father was now his conscious reason for hatred toward his father and his mixed sexual feelings later in life. Following this, the patient experienced a severe pressure on his right eye and reported seeing his father's penis. In response to the doctor's question, he stated that to keep the pain from his eye he could have directed the penis into his mouth. He doesn't want that now, although he did then.

This portrayal of an umbilical union with both parents, with its resulting psychosexual confusion might be readily delineated with psychological testing. It would be rare indeed that clinical interviews could reveal such basic dynamics so readily except through the therapist's intuition. It is unlikely that such meanings could purposefully be revealed to the patient so quickly. It is almost certain that the patient could not be induced to experience them in so undeniable a manner in so short a period of time.

In several cases, before and after the LSD episodes, interviews under narcosis gave us an opportunity to compare degree and character of cartharsis under these two influences. It was usually noted that the rate of verbalization in these cases and the unloading of feelings near the surface were greater and more rapid under narcosis, but the defense mechanisms were not shattered as with LSD. Hence, while the ventilation was usually found more continuous under sodium amytal-methedrine, under the disinhibiting influence of LSD the cartharsis was more significantly productive, more trauma-focused than under narcosis and the relief to the patient more reconstructive in its effect.

Intensity of Abreaction

The abreactions may be literal or symbolic; they may borrow from environment stimuli for a framework, they may transmute persons in the reality environment into neurotically cathected figures out of the past.

(One assisting therapist was strenuously choked during such a projective acting out.) The abreactions may be repeated over a number of episodes, each sustained for hours at a time, until the conflict so represented is resolved. Several patients, following episodes in which a conflict was abreactively experienced but not resolved, requested the drug the following day in order to go back and pick up the still entangled threads that wanted unraveling: "There's something else that wants to come out"; "something's still smothering me, I've got to find out what it is"; "I almost had an answer, but I wouldn't let myself go deep enough. I don't think I'll be afraid to let go this time." The drug, to these patients, was a means of "going back," a means of reexperiencing and, by so doing, of understanding present difficulties in terms of their past roots.

The abreactions were usually accompanied by a specific age awareness, sometimes only a sense of orientation, sometimes involving a marked alteration in body image as perceived physically by the patient.

One great benefit derived from this aspect of reaction to LSD was the patient's quick acceptance of the past as the source of his present adjustment difficulties. It made the meaning of the present maladaptation clear to the patient in terms of actual life experiences. It also gave a background of experience to the interpretations of the therapist and diminished the resistances customarily met in this process.

Effect on Motivation and Insight

The effect on motivation and insight is one of the more dramatic sequelae of LSD experience. This positive effect appears to result from the experiential aspect of the LSD reaction. The patient promptly finds that there are within himself feelings, meanings, memories, etc., of which he had been unaware, which he would have denied had the doctor simply told him they were there. In essence, he discovers his unconscious. One patient who was started on analytic therapy at the conscious associative level, ventilated, as if from under great pressure, intense resentment of his father. After three weeks of examining some aspects of this problem, he announced that he was more relaxed than he had been in years, and was ready to go back to work. Because resistance to looking at his mother was still insurmountable, LSD was recommended. The patient acquiesced, more to please the therapist than for any other apparent reason. His first episode under the drug took him literally and experientially back to his mother's grave at the time of her funeral. He grappled painfully with the chronic feeling of being lost and abandoned which he had been fighting since age 12 following his mother's death. He spontaneously admitted the following day that he had never believed the therapist's hints that he resented his mother for dying, but now that he experienced it, was willing to go on with therapy. He requested more LSD, despite its painfulness, because it permitted him to accomplish so much so rapidly.

Another patient, with a depressive anxiety reaction triggered by war stress, ventilated freely, although under considerable pressure, at the conscious level. After his third conference, anxiety associated with his verbalizations increased his discomfort and he asked about taking a drug to "block

out the past." The wish to forget was related to his wife's infidelity. Everybody could be made happy, he maintained, by forgetting the past. After eleven conference hours, during three week's hospitalization, the patient was sufficiently relieved of anxiety to consider himself ready for discharge. His improvement adjustment was largely repressive in nature and he could be encouraged to no insight into this development. Considerably against his will he accepted LSD and stated:

> "This is a shock. I thought I was well. I was going home tomorrow. I wanted to prove to mother, daddy, Margie, Johnnie, and all the others I wasn't as sick as they all thought I was. Now I don't know what to think. There is nothing I haven't told the doctor. I have all my problems licked and I am at peace with the world—I thought I had made so much progress and I am disappointed that Dr. Langner does not think the same way I do. . . ."

During the second episode, which was highly productive, the impact of the revelations relating to parental resentments were so disruptive that the patient was reluctant to take his planned weekend visit home. The false confidence stemming from previously reinforced repression was destroyed. He dreaded and desired the next experience under LSD. There was no longer any need to reinforce denial as the major mode of adjustment.

In the weeks devoted to LSD therapy, this patient experienced seven episodes. During his last episode, as the pressure of release was diminishing, he ventured that before LSD he had really thought he was well. Now, he no longer knew what to think. Seeing, through LSD experiences, how things "really were," he felt as though he had been "acting his life in a play." In retrospect, nearly two years later, when he answered the LSD questionnaire, he stated that LSD experiences "put me on my feet."

Patterns of Response

The types of response were as varied as the personalities of the patients responding. This brought under scrutiny in 1957 the popular misconception of the drug as a tool with which to induce a "model psychosis." LSD is a pharmacologically active substance, admittedly. However, this activity, rather than inducing something, appears to *release* variable somethings which are dependent on the individual's life experience and personality make-up. The only constant reaction noted in our hundreds of episodes was pupillary dilatation. In fact, this has been used to determine effective drug dosage. Other physical manifestations of the drug's presence were variable, and related only to the emotional state, chiefly on the sense of the defensive need to convert emotions to sensory and motor channels.

During our initial experiences with the drug, we were looking for patterns of response, and, in general, found none. While individual responses during certain stages of therapy were moderately predictable, as the therapy progressed, the patient's responses suggested a variety of rather different personalities—indeed, quite as different as the personality changes within the patient during their various redevelopmental phases.

For instance, one young woman initially felt intensely insecure, unworthy, hostilely dominated and split within herself; she felt that she had no real self. After a few episodes, this feeling of being split crystalized into a sharply focused, dramatic experience—theree voices trisecting her real life, her grandmother's, mother's and her own. Through the strength of her transference, she finally disengaged herself from parental and grandparental dependence and domination and cultivated a feeling of oneness —of *Self*—of being worthy and secure. The initial experiences had been cold, threatening and disruptive; the later experiences had been warm and reassuring. The changes in her Rorschach testing reflected a similar development. This represents a typical change in the pattern of response as the patient emerges from illness toward health.

PART TWO

Over 2,000 LSD sessions have been conducted under my supervision during the last three years. As a result of this additional experience, some new awareness and valuations have developed. Certain changes and elaborations in technique have led to some interesting observations. These comprise the subject of the rest of this paper.

First let us deal with the administration of the drug. I now prefer to do this after a minimum of two hours' fasting. With most patients I combine the dose of LSD, which ranges from 50 to 300 mcg, with 1½-3 grains of nembutal and 10 to 20 mg of methedrine. The purpose of the sedation is to diminish any anxiety that may create resistance. The methedrine maintains alertness and seems to provide a slight psychic push that would otherwise be absent. Thus, without trauma to veins, a combined narcosis-LSD effect is gained. In a number of patients this formula allowed the continuation of drug sessions, since LSD used alone resulted in disabling anxiety.

The average episode is still four to six hours. Before the patient is left alone, "termination" medication is usually given. This now consists of 100 mg Thorazine, or Mellaril, or 30-50 mg Librium which may need to be repeated in one to two hours. Rare panic reactions have been terminated by intramuscular Thorazine-barbiturate.

I would like to emphasize that patients are now both in- and out-patients. Carefully selected patients are capable of leaving the hospital after a treatment session eight to twelve hours after its inception. Patients permitted out-patient use of the drug must be carefully chosen and provided for. The possibility of dangerous acting out, should the patient "go back into the drug" after leaving the hospital *must be excluded in advance.* Severely depressed or hostile patients are not a safe risk for this type of management. Additional medication should be given immediately before leaving the hospital for the night; for this I employ 100 mg of Thorazine or Mellaril and carbital, grains 1½. This permits the patient's escort, usually the spouse, to put the patient to bed immediately on arrival home. Another condition necessary to out-patient use is the proper indoctrination of the spouse or other responsible family member. A definite prohibition

against permitting a patient to leave the hospital the same day is the lack of someone to chauffeur him and the absence of someone at home to supervise him during the night. Patients not meeting these conditions must remain hospitalized overnight.

The influence of various attendants and therapists came under scrutiny, since normally we would feel this to be of major importance. Actually this was not the case. Using the same attendant through a course of treatment, or rotating several attendants, or using attendants of different sex does not appear to have any decisive influence on the LSD situation, as long as the attendant does not interfere.

One manipulation that has proved useful is to introduce a second therapist with whom the patient has a prior therapy relationship, thus accenting in the immediate setting a parental duality. Another simple maneuver that may elicit feelings of value is to walk out on the patient who needs the attendant's presence as a shield for separation anxiety or as an external superego. Another means of stimulation that has been tried is the introduction of certain props at strategic times, such as a nursing bottle, dolls, objects to throw and to hit. In selected instances we find that patients who need the permissiveness and reassurance of an attendant may more quickly prepare themselves for acting out against the original object by being encouraged to do so first with symbolic representations of these objects, just as is done so often in the dream world. We do not focus these feelings directly on the therapist because when the very hostile patient acts out under LSD release, the symbolic target is in a position to get hurt. I have had three fairly aggressive attendants who had a need to promote interaction. Each one of these has been physically attacked.

During the past year, one innovation that has proved very rewarding has been to extend a principle I have used in individual therapy to LSD therapy. This is to have husband and wife involved together in the therapy of either one of them. My technique here is to start the interview with one spouse while the other listens. When the first spouse is ready to embark on LSD (where this is indicated), the spouse who is still in a listening status is allowed to sit in on the drug episodes, provided his or her ego and love are adequate to tolerate the productions of the spouse in treatment. When the original patient has received enough help to accept the entrance into therapy of the listening spouse, the former may be allowed, under adequate supervision, to act as "therapist" during the drug sessions of the second spouse. This not only reduces the cost of therapy, a practical consideration, but intensifies interrelatedness between them promoting a level of communication that few marriages enjoy.

In an attempt to formulate the proper role and value of the attendant during LSD episodes, the following conclusions were reached:

> The patient who feels intense anxiety under LSD will need some figure present who can offer adequate reassurance to avoid panic and permit the process to continue productively. The patient who is elaborating his feelings through somatizations can be helped to convert them to feelings by timely interpretation. The patient who

has come to a dead-end may occasionally be led into productive channels through diversion into neutral territory followed by a meaningful challenge. Many patients who are on the verge of acting out may be baited into so doing. However, in all these situations, if the support from the attendant has brought about reactions or productions that the patient is not ready to utilize, the episode may be followed by some degree of retreat, as in any therapeutic situation where feelings are provoked or reinforced before the patient is spontaneously ready for them. My over-all assessment is that the major purpose of the attendant or therapist during an LSD episode is to provide reassurance with a minimum of interference and with only occasional interpretations or challenges.

For a period of time I was quite curious about the effect of the expectation of the therapist on the LSD productions. This was accentuated by the reports of the Jungians who fostered the "transcendental experience." However, in reviewing this phase of the problem, I perceived that my patients had their share of transcendental experiences without my having so labeled them. But these transcendental experiences were not a zenith, except momentarily. The rapturous narcissistic self-affirmation that evolved from these episodes might carry the individual through a couple of days or a couple of months. In all instances, they fitted into the total therapy as a jumping off place, a place at which the patient had divested himself momentarily of the previously self-paralyzing negations and experienced a positive if regressive self-image from which further motivation to grow was crystallized.

Any patient may at some time in his therapy feel obliged to please the therapist or seek approval through pleasing. This is more difficult to do under LSD than when not under its influence. Many times, based on dream material or day-by-day reactions, one can point that the patient is ready to look at this or that phase of his problem under LSD intensification. The result is more variable than the conscious willingness or resistance of the patient. Some patients have consciously wanted to pursue a certain emotional channel but develop, during the episode, a phase of their problem wholly unrelated and even more basic and traumatic. Because of these observations, I feel that the expectation of the therapist influences the LSD production less than the developments associated with other types of therapeutic involvement.

As in all analytic therapies, patients under LSD have their defenses and resistances. A list of these is enumerated as a matter of interest. Dreamlike symbolization, especially hallucinations of foreign places—frequently oriental—and underwater scenes and hallucinations of geometric patterns are among the commonest visual projections. Somatizations are very common and eventually become receptive to interpretation. Vomiting and bodily cold are among the most common somatizations. Physical pain may be symbolic and defensive, or a matter of literal memory. Panic or disorganization are next in frequency. Euphoria without any productiveness is occasionally met. Narcissistic compensations of a dramatic and

elaborate nature have been encountered and are difficult to work through, especially in the schizophrenic. Sleep is very uncommon. Most of these defenses can be worked through over a period of time.

In 1957, results with LSD led to the hope of facilitating psychotherapy through a specific pharmacological agent. This hope was based on a concept fostered by LSD experiences, a concept I labeled "intrapersonal psychotherapy." In susceptible individuals, LSD was seen to bring the patient into contact with himself, breaking the barriers of time, converting buried memories into immediate experiences with the full impact of their meaning and influence on the frame of reference in which the patient now lived. Without diminishing the importance of the transference investment in the doctor, which permitted the patient's becoming involved with the therapy in the first place, it had to be admitted that much perceiving of meaning and working through of the conflicts was now going on, not in terms of a transference relationship, but in terms of the actual primary life relationship. Certain patients, through abreactions, were now able to relate their problems directly to the original interpersonal source rather than through a transference. Instead of gradually identifying feelings and their meanings in the transference relationship, certain patients went directly to the original relationship while under LSD and spontaneously experienced cause and effect, need and deprivation, hurt and anger, loneliness and fear, in their primal frame of reference and at the physical and emotional age level when they originally occurred.

This happened so effectively in some cases that the presence of the attendant or therapist was felt as a hindrance by these patients. They preferred to be alone during the episode to perceive the causal relationship without external hindrance. Others requested that the attendant be with them, but refrain from making comments or asking questions, desiring reassurance from the presence of another person, but no interfering interaction.

PART THREE

January, 1965. It has been four and a-half years since my last involvement with LSD as a therapeutic tool. The following opinions summarize my retrospective impressions of its hazards and values.

One phase of judgment, which has been the focus of much of my attention, is the degree of ego integration concomitant with abreactive experience under LSD. I conclude that it is often considerable, at least definitively more so than with hypnosis or narcosis in similar patients. With many patients, conference sessions between LSD episodes were devoted to the patients' spontaneous exposition of the meaning, past and present, of the most recent LSD episode. Such a sophisticated involvement was limited to the neurotics and the less regressed character disorders. In such patients the facilitating value of LSD still stands as a positive value in retrospective judgment.

Is the "working through" time diminished for the patient, as a result? We have no schedules or valid mass surveys to help us here. My personal

impression, however, is that LSD experiences somewhat lessen the expected working through time in patients with moderately strong egos. One reason may be that LSD provides the "crisis" which some patients need for self confrontation. For others it may be the ego-enhancement which fosters an on-going assimilation of material as it becomes conscious through abreaction.

Another area of value is in certain psychotics. My observations suggest that in a significant number an improvement in self-image and a lessening of unconscious regressive fixations are readily demonstrable. I am reminded of a manic-depressive alcoholic who had had manic episodes every one to two years during the thirteen years prior to my contact with him. He sweated and shook profusely through eight LSD episodes with practically no verbalization and conscious insight. He could say only that he felt relieved in an undefinable way after each episode. His follow-up during a three year period revealed improved relating, negligible moodiness and more stable performance than during his previous periods of repeated hospitalizations.

A teen-age schizophrenic who was disorganized, unable to attend school, suicidal and overwhelmed with guilt and unworthiness was another nearly silent LSD patient. She would spit on the floor during her first episodes, rub her face in her spittle and pantomine that this rejected saliva was herself. Five years after her hospitalization she was reported as successfully married and functioning creditably as a mother—with no evidence of relapse during this period. These are examples of cases where no post-hospital therapy was involved.

In general, it appears that the pseudo-neurotic and paranoid schizophrenics do not respond favorably. From my over-all experience with paranoid schizophrenics I feel that LSD has no value in this category and may, in fact, be dangerous.

Schizoid personalities whose ego is not too brittle may be benefited by the LSD experience insofar as it gradually gives some of them an experience with feeling. One schizoid summarized this to a group: "I know now that I never knew what people were talking about when they talked about feelings till I took LSD. I didn't know till toward the end of my second year of therapy that feelings could be good as well as bad."

Schizo-affective reactions with a severe depressive component were not dramatically effected by LSD in any of the several such patients exposed. I feel that there may be a real hazard in using the drug in the depressive psychotics due to both the overwhelming responses and the excessive withdrawal which may occur.

How would I employ LSD if I were using it currently? I believe I would welcome it to explore resistances in any patient with a good to moderate ego. I would expect it to be highly useful for short term therapy in acute neurotic decompensations or in character disorders where the development of anxiety signals failing defenses. I would find it useful for occasional strategic intrusion in the undifferentiated schizophrenic with marginal functioning.

Do I feel any patients are being denied an experience of significant

value as a result of non-acceptance of LSD as a therapeutic tool? Yes, I do. This was reemphasized this past summer by a 21 year old lad who had become disorganized the previous year, dropping out of college, working thereafter in a beatnik restaurant until he felt so empty he became practically immobilized. He drifted home in a state of helplessness and distrust, AWOL from Naval Reserve duty and threatening suicide if further military service were enforced. He went to New York in search of some undefined magic. While there he felt he was losing his mind. Black market LSD was offered by a friend who told him it might help. This lad summarized his one experience with LSD, when he saw me three months later. He had experienced his infantile need of woman. He perceived himself in terms of relationships past and present, acquiring some reality dimension of self. For two weeks after this single episode he had a sense of affirmation to his own being which was a new response within the limits of his conscious memory. Thereafter the euphoric phase diminished but just the memory of his experience that positive relatedness could be a reality provided motivation for new goals and lessened his previously devastating insecurity.

I cannot believe that such an instrument under proper supervision can be other than a useful chemotherapeutic agent in psychiatry. My experience with the drug in certain areas is positive. Further validation of its place in therapy is needed to promote any uniformity of opinion as to its usefulness, but there is already considerable evidence to suggest that the potential harm in the drug lies in its dramatic appeal to the sick therapist. I have wondered, further, if its repudiation by many is a function of too limited experience and, in some instances, the therapist's need to control the rehabilitative process more closely than can be done under LSD.